Goserelin 10.8 mg: hormone implant every 12 weeks

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

One implant every 12 weeks

Active medicines: Goserelin 10.8 mg

Emetogenic risk: Minimal

No routine prophylactic antiemetic order. Agree symptom treatment if needed.

Additional phase-specific support

- A trained healthcare professional places the implant under the skin of the front of the abdomen. This 10.8 mg implant has a different schedule from the 3.6 mg formulation; do not swap strengths or appointment intervals.
- Regular blood count, kidney and liver tests, blood glucose and bone blood profile. NCCP does not specify one interval for these tests. Review symptoms, PSA and testosterone, with scans according to the individual plan.
- Tell the team if you take blood-thinning medicines or are very thin. These can increase bleeding risk at implantation. Severe abdominal pain, marked swelling, dizziness or collapse afterwards needs urgent help. Do not stop blood thinners yourself.
- Bone health, calcium and vitamin D according to individual needs and local policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Sources checked 2026-11-14 · SACTCheck v0.84.52 · Regimen-level drafts; prescriber and oncology-pharmacy verification required